



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سعاد فتحي بيومي صقر
المحافظة : القاهرة
الحى / المدينة : عين شمس
رقم البطاقة : 27009070103841 رقم التليفون : 01156949718
العنوان : -----
العمر (تاريخ الميلاد) : 07-09-1970 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
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التوقيع: -----
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أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سعاد فتحي يومي صفة National ID: 27009070103841	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سعاد
فتحي بيومي صقر

١ ح محمود الجهنى ش. عيد ابى معق
عين شمس - القاهرة

٢٧٠٠٩٠٧٠١٠٣٨٤١

١٩٧٠/١٠/٢٧

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٢٠٢١/٠٧ ٢٧٠٠٩٠٧٠١٠٣٨٤١
بدون عمل

أنثى
مسلمة
متزوجة

اشرف ابراهيم عثمان عمر الملاوى
البطاقة مصرية حتى ٢٠٢٨/٠٧/٢٤

١



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / سعاد فتحى بيومى صقر

Patient No. : 27009070103841

Path No. : 717/25

Sex : Female

Age : 54 Y

Referring Doc :

Request No. : 13003837

Request Date : 19/02/2025

Verification Date : 13/03/2025

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:

* Procedure: Right MRM measured 20x15x5 cm with covering skin ellipse measured 17x12 cm.

* Tumor extent: Two retro-areolar and inner quadrant firm greyish white firm tumor bed sites.

* Size: 7x6x3 cm, and 3x2x2 cm

* Skin involvement: Nipple was fixed in one direction.

* Distance from margins: 0.7 cm away from least deep margin, and 0.1 cm from skin. The distance between 2 Tumor bed sites 2 cm.

* Lymph nodes number and size: 17 L.Ns up to 2.3 x1.5 cm.

* Designation: 717/25

1: Nipple + 1st tumor bed site.

2: Skin + 1st tumor bed site.

3-8: 1st tumor bed site.

9: Area in between 2 tumor bed sites.

10: 2nd tumor bed site.

11-12: 2nd tumor bed site + inked deep margin.

13-16: Lymph nodes.

Dissected by: Dr. Toka Medhat.

Microscopic Examination:

* Histological type: Bifocal invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

- Maximal tumor diameter 7 cm

- Actual tumor cellularity: 75%, tumor necrosis index: 25%

* Ductal carcinoma in situ: Focal DCIS 3%.

Verified by :

Dr.Toka Medhat





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / سعاد فتحي بيومي صقر	Request No. : 13003837
Patient No. : 27009070103841	Request Date : 19/02/2025
Path No. : 717/25	Verification Date : 13/03/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 54 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Angioinvasion: Positive.
- * Skin involvement: Infiltrated base of nipple with free epidermis.
- * Lymph node involvement: Positive lymph node metastasis in (5/17) lymph nodes.
- * Resection Margins: Free deep surgical resection margin.
- * Histologic response to therapy: Partial pathological response to therapy.
- * Microcalcifications: Positive.

Diagnosis:

- Right breast cancer post NACT, MRM specimen:
- Bifocal invasive duct carcinoma, grade II, with focal DCIS 3%, positive lymphovascular emboli and microcalcifications.
 - Maximal tumor diameter: 7 cm.
 - Actual tumor cellularity: 75%, tumor necrosis index: 25%.
 - Free deep surgical resection margin.
 - Free skin.
 - Positive lymph node metastasis in (5/17) lymph nodes.
 - Partial pathological response to therapy.
 - Stage: ypT3(m), ypN2.
 - RCB: 4.483, RCB class -III.

Signature
Prof. Dr. Lobna Omar
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 20/03/2025 12:07:47

Page 2 of 2

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patent Name: Gouan Fatty Polyol and Salt	Date: 02/07/2024
Exam: C. J. AGDOMENI & TILLYS	App: 94,394,154
Filed: 01/14/2024	Pat. ID: 85583

III The Examined Skeleton:

- D12 lytic osseous lesion measuring 2 cm.
- Heterogenous osseous texture of the iliac bones showing ill-defined lytic foci.

Opinion:

- Right breast neoplastic soft tissue lesions with metastatic right axillary lymphadenopathy.
- Right hepatic lobe segment VII small focal lesion.
- D12 lytic osseous lesion. For bone scan correlation.

Dr. Ali Elsisy, MSc.

Signature: Dr. Mohamed Elmaghrabi

13/07/2024 02:26



Patient Name: Saieda Cathy Dayana B58	Date: 08/07/2024
Exam: CT ABDOMEN & PELVIS	Age: 54 Years
Medical ID: 21472024	Pat ID: 86558

NON-CONTRAST MULTISLICE SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- Reported case of right breast cancer.

I Chest:

- Right breast parenchymal soft tissue lesions, the largest is seen inseparable from overlying skin measures about 4.3 x 4.7 cm in axial dimensions and smaller one is seen merging with the underlying pectoral muscles. Diffuse skin thickening is also noted.
- Associated pathologically enlarged right axillary level I lymph nodes, largest measures about 1.9 x 3.2 cm.
- Few scattered bilateral small pulmonary nodules (3-4 mm).
- No pulmonary areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is average in size showing right hepatic lobe segment VII small subcapsular hypodense focal lesion of gross fatty attenuation measures about 6 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Prominent mesenteric, para aortic and aorto caval lymph nodes, largest mesenteric node measures about 1.2 x 1 cm.
- No ascites.



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : سعاد فتحى بيومى صقر

Sex:

3- Medical number:
Female

83217373

Old Med :

Old Med :

2141-2024

2-ID number: 27009070103841

Patient Age : 54 Y



Family history

Family History
-ve

Phone

Phone
01156949718

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

Marital Status
MARIED

NRS Score

NRS Score

GROUP

GROUP
C

Date of MDT

Date of MDT
29/07/2024

Co morbidity

Co Morbidity
-veMenopausal
status

Menopausal Status

Initial STAGE

Initial STAGE
Advanced

Clinical data

Breast Imaging :

rt breast multicentric mass at 12 oclock with retracted nipple & invading
4.5 , 4.3 + LN

Mets Workup :

rt hepatic lobe VII small focal lesion D12 lytic lesion

BX

IDC

BX COMMENT:
rt il

ER

8/8

ER COMMENT:

PR

7/8

PR COMMENT:

Her 2 Neu

1

Her 2 Neu COMMENT:

SISH

SISH COMMENT :

Ki67

20

T

cT2

multicentric

N

2

M

?bone
liver

Sonomammography

PATIENT HISTORY

PET-CT (EQUIVOCAL RESULTS ON CT)

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR NACT TAXOTERE X4 THEN ACX4 THEN MRM (IF M0)

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

SURGERY

SURGERY

SIGNATURE

RADIOLOGY

RADIOLOGY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Name : Soad Fatthy Bayoumi Sagr
Branch : Maadi
Ref. Physician Prof : Goustav Roussey Egypt

Date : 19-06-2026 01:19
Medical No. : 397491
تعاقد مستشفى : نيفك

Tc-99m MDP SKELETAL SCINTIGRAPHY

History:

56-year-old female patient with history of surgically treated right breast cancer received chemo-radiotherapy and hormonal treatment complaining of bone aches and referred for skeletal survey.

Technique:

Anterior and posterior whole-body scans 2 hours post injection of 20 mCi of the tracer.

Findings:

The surveyed skeleton is the seat of multiple, randomly distributed foci of variable tracer uptake keeping with bone secondaries. They are located at the high parietal region of the skull bone, proximal part of the right humerus, sternum, bilateral scattered ribs, most of dorso-lumbar vertebrae and pelvic bones.

IMPRESSION:

- Metastatic bone disease at the aforementioned sites.
- Follow up bone scan is advisable to monitor response to therapy.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

د. جلاء Nagy.
مفاد

Patient's Name:

Soad Fathy Bayoumi Sagr

Date:

2026-06-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.
ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent right MRM.

I- CHEST:

- Right upper lobe subpleural reticulations likely post radiation sequelae.
- Few bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or caviary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II-ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. Multiple bilobar well defined hypodense focal lesions with peripheral contrast uptake, largest measuring 16 x 8.8 mm at segment V of the right hepatic lobe. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular lytic bone lesions with extra osseous soft tissue components seen at D5 vertebra and the left iliac bone.

Opinion:

Patient's Name:

Soad Fathy Bayoumi Saqr

2026-06-24

Date:

Prof.Dr. no doctor

Referred by:

Goustave Roussey International Egypt

Hospital:

- ❖ Multiple bilobar hepatic focal lesions likely metastatic for dynamic MR study.
- ❖ Multiple axial and appendicular skeleton lytic bone lesions with D4 vertebral and left iliac bone extra osseous soft tissue components.

Dr.sherif W agdy

د. شريف واعدى

Patient's Name:

Date:

Referred by:

Hospital:

Soad Fathy Bayoumi Saqr
2026-06-22

Prof.Dr. no doctor

Goustave Roussey International Egypt

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Two pathological lymph nodes are seen, one at level II measures 12x7mm other at infra-clavicular measures 11x6mm

Conclusion:-

- **Left breast (BIRADS 1): normal study.**
- **Two pathological lymph node at axilla for further assessment**

Recommendation:-
For annual follow up.

Best Regards,
DR. Marwa M.Hassan

Patient Name MONA FADALY	
Exam : Ultrasound ABDOMEN& PELVIS	Date : 22/06/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (14cm at Rt MCL) displaying heterogenousparenchymal echo pattern .
- No IHBRD seen
- Portal vein is patent and not dilated.

Gall Bladder: - no stones visualised

Spleen: -

- Average size 15 cm, homogenous texture with no focal lesions

Kidneys: -

- kidneys of normal size, shape,echogenicity
- No stones , no back pressure changes

Para-aortic and pancreatic area: concealed by gaseous distention

Urinary Bladder: -

- Minimally filled no stones or masses

Uterus:

No visualized masses

Opinion:

Unremarkable study

Name : Soad Fatthy Bayoumi Sagr
Branch : Maadi

Ref. Physician Prof : Goustav Roussey Egypt

Date : 19-06-2026 01:19

Medical No. : 397491

شركة مستشفى : نيل
Ins. Company :

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IMPRESSION:

- Metastatic bone disease at the aforementioned sites.
- Follow up bone scan is advisable to monitor response to therapy.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

د. جلاء Nagy.
مفاد ردة

Patient's Name:

Soad Fathy Bayoumi Sagr

Date:

2026-06-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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Opinion:

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2026-06-24

Date:

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Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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Dr.sherif W agdy

د. شريف واعدى

Patient's Name:

Date:

Referred by:

Hospital:

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2026-06-22

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